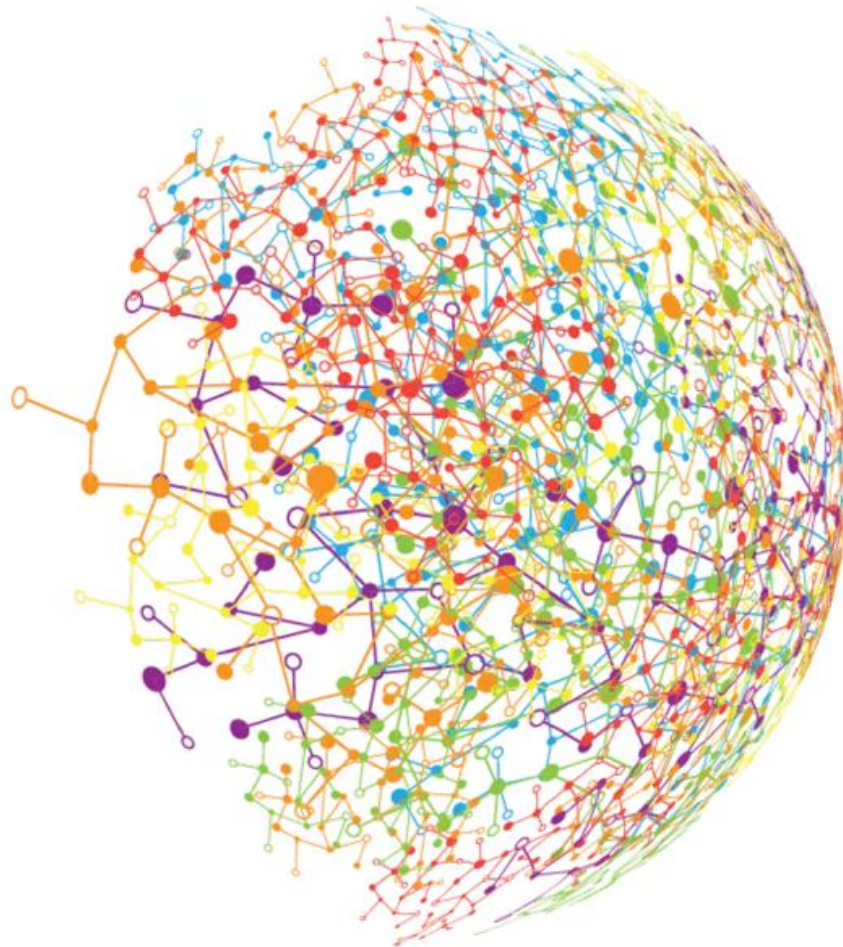




Vermont Agency of Human Services Office of Health Care Reform

Hospital Transformation Analytical Support

October 28, 2025

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HOSPITAL TRANSFORMATION ANALYTICAL SUPPORT PROPOSAL

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1 COVER LETTER

October 28, 2025

Matthew Rockwell
Agency of Human Services – Office of Health Care Reform
280 State Drive
Waterbury, VT 05676

Re: Hospital Transformation Analytical Support- Request for Proposal

Dear Mr. Rockwell,

Deloitte Consulting LLP (“Deloitte”) is pleased to submit this proposal in response to the *State of Vermont’s Request for Proposal: Hospital Transformation Analytical Support*. We appreciate the opportunity to partner with the Agency of Human Services (AHS) and the Office of Health Care Reform to advance Vermont’s (“the State”) commitment to sustainable, high-quality, and efficient health care delivery.

Deloitte’s ability to deliver timely, strategic analysis for Vermont stems from the unique combination of our national knowledge and experience in hospital analytics and transformation modeling and our hands-on familiarity with Vermont’s data landscape. Our team has led similar engagements across the country helping states and health systems model transformation scenarios, benchmark performance, and inform policy decisions that balance access, quality, and cost. At the same time, we bring unmatched insight into Vermont’s own data environment through our ongoing work managing the State’s Medicaid Data Warehouse and our extensive understanding of its privacy, security, and governance frameworks.

Because of this dual perspective, Vermont can expect rapid mobilization and minimal onboarding. Our public-sector health transformation experts supported by accountants, clinicians, data scientists, and policy strategists are prepared to begin immediately, delivering the analytical modeling, benchmarking, and scenario planning envisioned under this RFP. Our combined national and local knowledge and experience position us well to turn data into actionable insights that help the State chart a sustainable, high-performing health system for all Vermonters.

We are inspired by Vermont’s leadership in creating a sustainable healthcare ecosystem and look forward to supporting this important next step in your transformation journey. Please contact me directly with any questions regarding this submission. If you have any questions or would like to discuss our response further, please contact me at gspino@deloitte.com or by phone at +1(617)-448-9688.

Sincerely,



Gregory Spino, Managing Director

Deloitte Consulting LLP

2 EXECUTIVE SUMMARY

2.1 Our understanding of the problem

Vermont (VT) seeks to find a sustainable path forward for its hospital and health care systems, preserving equitable access to essential care while supporting the long-term financial stability of its healthcare providers. Across the state, rising costs, declining margins, and workforce shortages are putting increasing pressure on hospitals. At the same time, Vermonters in rural and remote areas continue to face barriers to timely access for specialty and emergency services. These intersecting challenges have placed the system at a crossroads: how to maintain access to high-quality care for every community in a financially sustainable way.

The State of Vermont seeks a partner to provide advanced analytic and modeling support that informs hospital and regional transformation planning under Acts 167 and 68¹ and aligns with the Centers for Medicare and Medicaid Service's (CMS) Rural Health Transformation Program. Vermont's objectives are to evaluate reforms that balance access, quality, and financial sustainability while reducing statewide healthcare costs, to build transparency, and to facilitate evidence-based policy decisions. The goal is not merely to assess current operations, but to test future configurations of hospital and regional service networks that balance cost efficiency, quality, and equitable access. This means identifying opportunities for collaboration across regions, understanding which services are essential to maintain locally, and determining where regional models or innovative delivery systems—such as Hospital-at-Home or telehealth—can strengthen both access and financial resilience.

Our approach can achieve this vision and is designed to:

- Integrate multi-sector data into an analytic foundation for hospital and regional planning
- Deliver agile, iterative scenario modeling that enables “what-if” analyses of transformation options
- Support data-driven, transparent decisions by the Vermont Agency of Human Services (AHS), Green Mountain Care Board (GMCB), and other relevant stakeholders
- Produce publicly interpretable dashboards and reports aligned with Vermont's Snowflake and Tableau environments

This approach gives Vermont the tools to respond to the health care realities Vermonters face every day. Across the state residents are experiencing rising health care costs, limited access to specialty and emergency services, particularly in rural areas, as well as growing concerns about the long-term viability of community hospitals. These challenges have led to difficult trade-offs for families, providers, and policymakers alike.

By combining comprehensive data integration with scenario modeling, this transformation scenario work will enable the State to make informed choices about how to preserve essential services, stabilize struggling facilities, and direct resources where they will have the greatest impact. Vermonters will benefit from a system that is innovative, financially sustainable, regionally coordinated, and centered on equitable access to high-quality care so that whether they live in Burlington, Bennington, Newport, or the Northeast Kingdom, they can count on receiving the care they need.

¹ Act 167 (2022) and Act 68 (2023) of the Vermont General Assembly direct the Agency of Human Services and the Green Mountain Care Board to develop and implement a hospital transformation strategy to facilitate sustainable access to essential health services across the state. These Acts require coordinated planning, financial and operational analyses, and stakeholder engagement to support system-wide reforms that balance quality, access, and affordability with a focus on Vermont's rural and community hospitals.

2.2 Overview summary of our approach

To tackle the most pressing issues to Vermonters, our team’s collaborative and phased approach provides insights for AHS in distinct stages to develop a state-wide transformation roadmap that guides decisions balancing access, quality, and financial sustainability. By bringing a combination of our national knowledge and experience in assessing hospital resiliency as well as our team’s depth in understanding hospital and health system data, our team is ready to begin day one without delay. Our four-phase approach provides Vermont with a clear, data-driven roadmap (inclusive of integrated financial and operational analytics, facility risk profiles, scenario-tested recommendations, and stakeholder-ready reporting tools) that guides hospital transformation and drives long-term system sustainability. We have utilized this proven approach effectively in similar projects with other states.



- **Phase 1** establishes a strong analytic foundation by aggregating and standardizing data across all payers including the All-Payer Claims Database (APCD)², Vermont Health Care Uniform Reporting and Evaluation System (VHCURES), and, as needed, Deloitte’s CMS Cost Report database, and Vermont Medicaid data currently housed in the Medicaid Data Warehouse. This foundational work will produce a unified, high-quality data environment and baseline utilization report that gives the State its first comprehensive, comparable view of hospital performance.
- **Phase 2** utilizes advanced modeling to dive deeper into hospital performance and understand service volume capacity regionally and statewide. This analysis will provide a deeper understanding of hospital performance and service volumes to uncover possible collaboration opportunities.
- **Phase 3** transforms the analysis into tangible options by modeling different scenarios that contemplate alternative access points, care delivery models, and policy or regulatory changes. These insights will identify where Vermonters seek hospital care, where services overlap or are lacking, and where opportunities exist for regional collaboration and efficiency. Interactive dashboards, scenario modeling, and facilitated stakeholder engagement will empower Vermont’s health leaders to make informed choices about preserving access and strengthening regional networks.
- **Phase 4** connects financial viability and strategic planning into a statewide transformation roadmap. This roadmap will provide data driven recommendations to balance access, quality, and cost across the health ecosystem. Together, these phases deliver the data, insights, and collaborative framework designed to help Vermont build a resilient, equitable, and sustainable hospital ecosystem that meets the needs of all Vermonters.

The table below provides an at-a-glance view of Deloitte’s four-phased technical workplan. It highlights the purpose, data progression, analytical outputs, and decision-making insights generated at each stage, showing how Vermont’s hospital transformation effort moves from foundational analysis to actionable statewide policy guidance.

² Our understanding is VHCURES has 60% of commercial volume and 100% of Medicaid and Medicare volume. As such, accessing VHCURES is critical to project success. We will prioritize advancing any required Data Use Agreements to access VHCURES, and, if needed, Deloitte can bring Medicare data to combine with Medicaid to kick-start analysis while VHCURES access is being arranged.

3 TECHNICAL APPROACH AND WORKPLAN

Table 1: Summary of Proposed Workplan				
	Phase 1 (Months 1-2): Data Integration & Foundational Analysis	Phase 2 (Months 3-5): Deeper Insights into Hospital Performance & Services	Phase 3 (Months 6-7): Regional Transformation & Scenario Exploration	Phase 4 (Months 8-9): Policy Levers & Opportunities
Description	Build a unified analytic foundation that knits together multiple data sources and establish baseline hospital performance metrics	Assess financial performance and regional service capacity to identify collaboration opportunities	Translate data into actionable regional scenarios to test options and stress-test system sustainability	Integrate results into a statewide roadmap linking analysis to policy levers and transformation actions
Data Sources	- VHCURES - CMS Cost Reports, Medicaid Data Warehouse* - Snowflake/Tableau Environment - PeoplePrism (as needed)	- VHCURES - CMS Cost Reports, Medicaid Claims Data* - Quality Data - Select Neighbour-State Facility Data (NY, NH, MA)	- VHCURES - CMS Cost Reports - Hospital Financial Statements - Quality Data - PeoplePrism	- VHCURES - Audited Financial Statements - PeoplePrism - State and Federal Policy Inputs
Insights Generated	- Clear, validated understanding of hospital performance across Vermont - Foundation for data-driven transformation planning	- Regional view of financial and operational strengths and weaknesses - Insight into migration patterns, duplication, and service gaps	Evidence-based scenarios that quantify trade-offs in cost, access, and equity; support for regional planning and resource prioritization	Actionable statewide policy options and investment priorities; cohesive roadmap guiding sustainable and equitable transformation
Deliverables	- Data Integration and Analysis Plan - Baseline Utilization and Financial Performance of VT Hospitals	- Market and Financial Analysis - Evidence-Based Benchmarking Evaluation	- Hospital & Regional Access Models - Transformation Scenario Model - Ideal Scenario Models	Final Regional and Hospital Transformation Analytics Report (Statewide Transformation Roadmap Report)

Table 1. Overview of the four-phased technical workplan for Vermont's Hospital Transformation

3.1 Phase 1: Data Integration and Foundational Analysis (Months 1-2)

In Phase 1, the team's primary focus is on data aggregation, standardization, and preliminary analysis to establish a unified analytic foundation for the project. A key element of this effort is gaining access to VHCURES and

integrating it with other critical datasets. Gathering and harmonizing data across all payers requires multiple state approvals, but this process is essential for establishing accurate, comprehensive insights into hospital utilization and financial performance across Vermont and neighboring states. This work will provide the State of Vermont, AHS, and GMCB with a continuously refreshed, comparable baseline for understanding hospital sustainability and transformation readiness.



A key focus of Phase 1 is establishing leading indicators to help the State proactively identify at-risk hospitals and emerging operational pressures. Early warning metrics include cash-on-hand, operating margin trends, service line utilization, and staffing metrics. These indicators enable Vermont to transition from retrospective review to predictive management, aligning with the goals of Acts 167 and 68. Any prior work or existing analyses relevant to Vermont's hospital system, including previous data integration, claims studies, or operational reviews, can be leveraged to accelerate progress, enhance the analytic foundation, and support interpreting historical data to drive forward-looking transformation.

As needed, our team's pre-built Medicare Cost Report data warehouse allows preliminary analysis to begin immediately as VHCURES access and integration are finalized, minimizing startup time. We are able to leverage the existing Medicaid Data Warehouse and Analytics Solutions (MDWAS) Data Warehouse environment, including Snowflake and Tableau, and these new data will be ingested and integrated as needed to support initial analysis. The result is a temporary foundation to support initial analysis, as needed until VHCURES is accessible.

Deliverables

- **Data Integration and Analysis Plan:** Plan that documents, ingests, and synthesizes all data sources, logic and integration methods, and defines key utilization metrics such as occupancy rates, discharge volumes, and services line distributions. This plan will incorporate standards for network adequacy and measure proposed transformation relative to standards. Lastly, the plan also begins to identify opportunities to align resource allocation with resource needs.
- **Baseline Utilization and Financial Performance of VT Hospitals:** Summary of collected and cleaned data illustrating early indicators to identify potential at-risk hospitals.

3.2 Phase 2: Deeper Insights into Hospital Performance & Services (Months 3-5)

Rapidly progressing, Phase 2 builds upon the integrated data foundation that the team established in Phase 1 to conduct hospital and service line analysis. This phase combines service utilization with financial performance analysis to create an actionable picture of Vermont's healthcare delivery landscape. This phase's goal is to assess regional service capacity and quantify opportunities to improve access, equity, and financial sustainability. To map out clinical services volumes for acute care facilities in Vermont and contiguous states to refine scenarios, the team can extend Medicare Cost Reports analysis to hospitals in neighboring states including New York, New Hampshire, and Massachusetts, to compare service offerings and volumes as Vermonters may access these facilities due to their geographic proximity³. The team can leverage VHCURES data to conduct a deep dive into inpatient utilization

³ Note: Contiguous state analysis is limited to Medicare as a proxy for all-payers as out-of-state data are not available through VHCURES or other Medicaid data. Medicare data will be accessed via Deloitte's existing Medicare data warehouse.

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by service line and begin migration analysis with Medicaid population and service line groupings to understand utilization and migration patterns in Vermont. Based on the data, the team can identify unique or missing services by region; highlighting areas of overlap or deficiency under various scenarios benchmarking against national CMS, Leapfrog, and Hospital Compare measures. In tandem, the team can develop a service distribution dashboard including Vermont and proximate facilities in New York, New Hampshire, and Massachusetts and visualize a comparative analysis report using the insights the team gathered.

In addition to the proposed price for this project, Deloitte can provide organizational change management as well as operational, financial, and clinical assessments to hospitals across the state. These performance improvement efforts at individual hospitals will help translate the insights from statewide data assessments into tangible improvements in hospital outcomes similar to work we have done in other states.

Deliverables

- **Market and Financial Analysis:** Report that benchmarks Vermont hospitals against regional and national standards for cost, quality, and access. These reports include a detailed market analysis of Vermont hospital facilities including financial, quality, and clinical performance.
- **Evidence-Based Benchmarking Evaluation:** The evaluation includes a framework outlining key metrics, timelines, and performance indicators for implementing regional transformation strategies.

3.3 Phase 3: Regional Transformation and Scenario Exploration (Months 6-7)

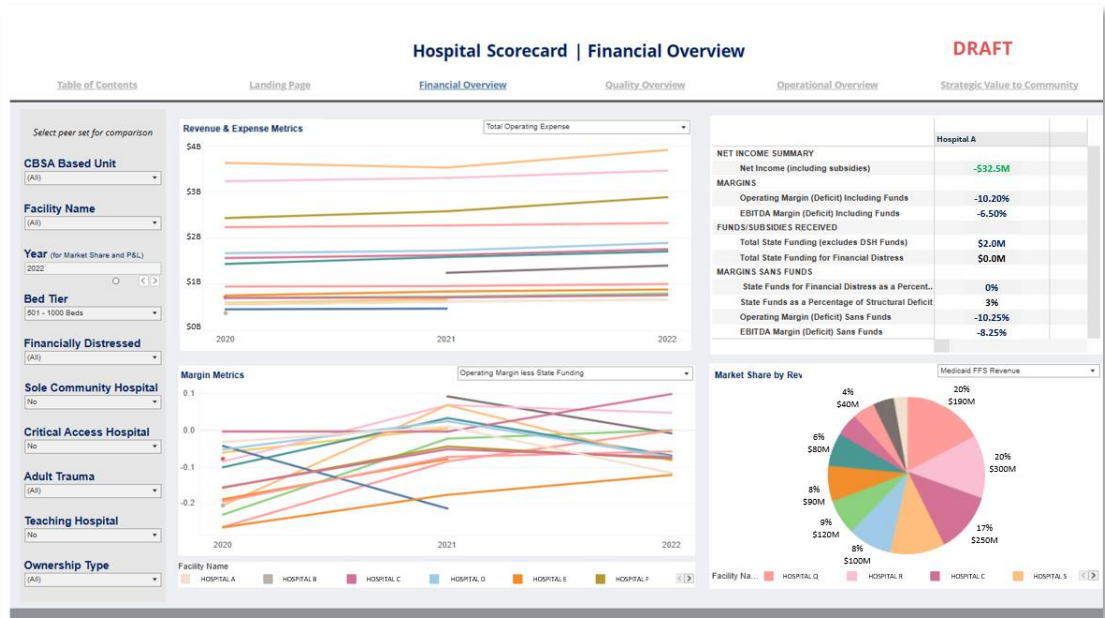
In Phase 3, our team can transform Vermont's analytic insights into a series of actionable regional transformation models that our team has designed to inform policy, funding, and hospital planning decisions. In this phase, we plan to translate the detailed utilization, access, and financial data from Phases 1 and 2 into structured, scenario-based analyses that evaluate hospital and regional sustainability under multiple future-state configurations. Our objective will be to identify viable, evidence-based pathways for regional collaboration, service optimization, and system-wide financial resilience.

Phase 3 begins with an analysis of hospital inpatient utilization data and patient migration patterns resulting in a deep understanding of where and how patients use services and migrate across the region to specific facilities. This in-depth data analysis and visualization will incorporate quality of care metrics, operational KPIs, and acuity information to assess the performance of individual hospitals. Our team can also utilize Vermont Medicaid claims data to glean insights around patient origin and migration patterns to understand to which facilities and areas patients are traveling to and for which services. As new data becomes available, our team can expand the analysis to include procedural care and eventually all service types. This analysis will also use VHCURES for a comprehensive view of utilization across payer types. Lastly, the team can create interactive dashboards for patient flows and utilization hotspots that will allow the user to drill down by payor type and service line to identify migration patterns.

To translate these analyses into actionable insights, our team can develop interactive dashboards that combine financial, operational, and utilization data for each Vermont hospital. These visualization tools can empower state and hospital leaders to identify at-risk facilities and understand the clinical, operational, and financial drivers at an individual facility, within a health system, or across the state. Figure 1 illustrates an example of this type of dashboard, highlighting how policy makers can visualize and interpret key performance metrics to make faster, more informed decisions that balance financial stability with high-quality, equitable care.

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Figure 1: Illustrative Dashboard Financial Overview



This is an example hospital scorecard that would be included in the hospital utilization reports.

Dashboard Functionality

This view allows state leaders to assess financial performance trends and market share dynamics in one integrated dashboard. On the left, users can analyze key financial metrics such as revenue, expenses, and operating margins over time. On the right, a profit and loss summary and revenue-based market share visualization provide an at-a-glance understanding of comparative performance across facilities and systems

Data Source	VHCURES
Key Questions Answered	➤ How have selected facilities performed over time across critical revenue, expense, and margin metrics? ➤ What is the current financial standing and revenue-based market share of a given facility or health system?
Value added	This dashboard enables state leaders to link financial performance with patient access and workforce trends in one view. For example, hospitals can identify where declining margins and volume point to opportunities for regional collaboration instead of costly standalone expansions, or where rising labor costs and margin pressures indicate a need to redesign staffing models or pursue telehealth partnerships. In contrast to past decisions driven by retrospective cost reports or anecdotal information, this tool delivers timely, evidence-based insights that help reduce costs, strengthen sustainability, and protect patient access.

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Throughout this phase, our team could use scenario-based modeling to stress-test regional strategies under different policy and reimbursement environments. The scenarios test how reimbursement changes and policy reforms may influence hospital margins, workforce stability, and patient access. These insights can guide AHS, GMCB, and hospital leaders in prioritizing interventions that balance cost savings, quality improvement, and equitable access.

Our team can begin the scenario planning process by pinpointing opportunities for regional collaboration and service optimization through a data-driven analysis of service distribution and alignment. Crucially, to build buy-in and stakeholder alignment, our team can facilitate up to 3 Regional Transformation Workshops with AHS, GMCB, and hospital stakeholders. These sessions will present quantitative findings, validate scenario assumptions, and support consensus on next-step priorities. The workshops will focus on how the newly uncovered insights relating to patient migration and service distribution can and should inform real-world decision making. With a clearer picture of why patients migrate for care and which facilities and services they seek, the focus will shift from data analysis to real-world implications. The discussions can consider various approaches for service alignment and optimization, including pathways like Hospital-at-Home programs, broader telehealth use, regional service line adjustments, shared staffing models, and focused workforce investments.

The collaborative structure allows decision-makers to visualize how regional configurations perform under varying financial and clinical pressures, establishing a clear link between analytical findings and actionable policy choices.

Deliverables:

- **Initial Regional and Hospital Transformation Analytics Report:** Materials designed to frame the workshop are provided prior to the workshop to allow all participants time to prepare, and follow-up summaries including detailed notes as well as next steps and action items.
- **Hospital & Regional Access Modeling:** Detailed dashboards featuring interactive data visualizations that summarize hospital specific data (financial, operational, and quality) as well as region-wide data, including service-line volume, patient migration patterns, and access metrics across Vermont and neighboring regions (proximate locations in NH, MA, and NY based on Medicare Cost Report data) to better understand both in-state and out-of-state services. These dashboards will allow for an integrated comparison between high-performing and low-performing facilities across the state and an in-depth view of region-wide patterns and trends.
- **Transformation Scenario Model:** Drawing on national and Vermont-specific data outlined above, the models feature a set of 'ideal state' scenarios for the regionalization of specific service lines that align with statewide care transformation goals and assess proposed service line and hospital changes in the context of the 'ideal state' scenarios, highlighting alignment, gaps, and potential trade-offs. Models also consider and include any impacts of new, innovative delivery models (potentially including but not limited to hospital-at-home, telemedicine, EMS/EMT triage, transfer and transport enhancements, interoperability).

3.4 Phase 4: Policy Levers and Opportunities (Month 8-9)

In Phase 4 we develop Vermont's statewide transformation roadmap. This roadmap serves as Vermont's strategic guide for achieving a financially sustainable, equitable, and high-performing hospital system. Building on the data and insights from Phases 1-3, Phase 4 will deliver crucial evidence and financial analysis to support sustainable planning across the state including innovative delivery models. The roadmap will combine evidence-based analytics, financial viability assessments, and policy recommendations to support system-wide alignment across the Agency of Human Services (AHS), the Green Mountain Care Board (GMCB), and other key stakeholders.

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Our team can conduct a statewide financial viability assessment that integrates hospital audited statements, CMS Cost Reports, and VHCURES. This analysis will link patient utilization and care delivery patterns directly to the financial health of hospitals, identifying systemic risks and opportunities for innovation. Facility-specific risk profiles will be developed to highlight strengths, vulnerabilities, and pressures such as declining margins, workforce shortages, or service duplication. These profiles can inform resource prioritization and guide where stakeholders could implement targeted interventions, such as shifting services, closing facilities, or transferring services. The financial analysis work will validate that the transformation roadmap aligns with Vermont's needs, balancing bold and necessary changes in operations with feasibility.

The team will use scenario-based financial and operational models to assess how reforms like service line changes, workforce investments, or technology adoption impact financial sustainability, care access, and quality outcomes. These stress tests will help Vermont identify the optimal balance between fiscal responsibility and health system resilience.

Key transformation areas will include:

- **Innovative Delivery Models:** Evaluating and modeling the impact of micro or neighborhood hospitals, Hospital-at-Home, tele-Eds, telehealth, mobile health, and Artificial Intelligence (AI)-driven innovations. These models will be analyzed to determine their effects on access, care quality, cost reduction, and workforce efficiency, especially in rural and underserved regions.
- **Workforce and Infrastructure Optimization:** Assessing strategies to stabilize Vermont's health workforce and align infrastructure investments with future care delivery needs.
- **Payer and Policy Alignment:** Analyzing how shifts in reimbursement models, Medicaid and Medicare policy changes, or potential federal waivers could strengthen Vermont's transformation goals.
- **Equity and Access Planning:** Ensuring that proposed models preserve equitable access for vulnerable populations and align with Vermont's commitments under Acts 167 and 68.

To demonstrate the tangible impact of innovation, our team can incorporate a detailed Innovation Scenario Modeling Framework that evaluates the operational and financial viability of emerging care delivery models.

Limited to two innovation scenarios, real-world Vermont data could be used to model variables such as:

- Changes in cost of care, operating margin, and revenue diversification
- Improvements in geographic and demographic access to services
- Effects on care efficiency, such as reduced readmission rates and optimized staffing utilization
- Workforce productivity gains and reductions in administrative burden
- Advancements in quality and patient outcomes driven by technology-enabled care

This transformation effort centers on equitable access and improved health outcomes for all Vermonters.

Quantifying access and outcomes by geography, income, and demographic group under varying transformation scenarios will guide decisions that advance fairness and align with Vermont's commitments under Acts 167 and 68.

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Vermont's access to the PeoplePrism platform, combined with Deloitte's extensive experience applying its analytic architecture, will further strengthen this equity and access modeling. Integrating PeoplePrism's comprehensive demographic, socioeconomic, and geographic data with VHCURES, enables the State to gain a more complete understanding of how community and social factors influence hospital sustainability, access, and outcomes. These insights highlight disparities in service availability, workforce capacity, and financial vulnerability, enabling Vermont to target interventions where they make the greatest impact. Leveraging this resource allows the statewide transformation roadmap to reflect operational and financial performance and the lived realities of Vermonters, helping to build a system that is equitable, resilient, and sustainable.

The following table outlines how each transformation area will be analyzed through the Innovation Scenario Modeling Framework. By incorporating Vermont's access to the PeoplePrism platform, the framework expands traditional financial and operational modeling to include community-level demographic and social determinants of health insights, providing a more complete foundation for data-driven policy decisions.



DELOITTE

PeoplePrism™

PeoplePrism™ is an advanced analytics solution developed by Deloitte to support public programs. By integrating data from more than 300 public and private datasets from 30+ vendors, including from DataUSA, **PeoplePrism** identifies and segments populations based on 1,700+ social drivers (SD) data points from a vast database containing over 270 million population records spanning across 10 years. PeoplePrism also leverages advanced AI/ML modeling techniques and maintains 150+ health behavior models in addition to 15 field-tested marketing personas.

The data sources in **PeoplePrism** are aggregated and refreshed monthly so that the insights provided are current and relevant. The granularity of the data allows for exploration of populations at county, ZIP code, Census Tract and Census Block geographic levels.

Compliance with regulations is maintained using de-identification methods, regular audits, and the use of ethical AI practices. This provides privacy and security of the data processed by **PeoplePrism**.

Table 2: Key Transformation Areas and Innovation Scenario Modeling Framework			
Transformation Area	Analytic Focus	Example Analyses and Modeling Activities	Intended Outcomes for Policy and Planning
Innovative Delivery Models	Evaluate emerging care models including Hospital-at-Home, Telehealth, mobile health, and AI-enabled operations	Simulate operational and financial performance of technology-enabled care delivery using Vermont data; assess care efficiency, workforce, and cost impacts	Inform reimbursement reform, infrastructure investment, and statewide adoption of high-value care innovations that improve access and efficiency
Workforce and Infrastructure Optimization	Assess strategies to stabilize Vermont's clinical workforce and optimize use of physical and digital infrastructure	Model facility utilization, staffing ratios, and workforce demand under different transformation scenarios to identify sustainable staffing and facility needs	Guide investments in workforce retention, training, and facility modernization aligned with future service demand and care delivery innovations
Payer and Policy Alignment	Analyze how reimbursement, Medicaid and Medicare policy shifts, or waivers affect sustainability and access	Model impact of different payment structures and policy levers on hospital margins, access, and quality across rural and urban settings	Support data-driven design of payment and policy reforms that align incentives with Vermont's transformation and equity goals

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Equity and Access Planning	Advance equitable access and health outcomes across Vermont's hospital system Identify and address disparities in care availability, quality, and financial stability; align transformation investments with communities experiencing the greatest need; support data-driven policy decisions consistent with Acts 167 and 68	Integrate PeoplePrism's demographic, socioeconomic, and geographic data with VHCURES, CMS Cost Reports, and hospital financial data to analyze access, workforce distribution, and outcomes by region and population group	Identify and address disparities in care availability, quality, and financial stability; align transformation investments with communities experiencing the greatest need; support data-driven policy decisions consistent with Acts 167 and 68
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Table 2. Key Transformation Areas and Innovation Scenario Modeling Framework

Outputs from this modeling will include quantitative impact reports and interactive dashboards displaying key metrics across innovative domains. These results will provide AHS and GMCB with a transparent, data-backed understanding of the benefits, trade-offs, and feasibility of various innovation pathways.

The roadmap will also include a sustainability framework that aligns operational improvements, financial strategies, and policy actions into a cohesive plan. Our team can work with AHS and GMCB to co-develop a Transformation Sustainability and Handoff Plan, supporting Vermont to maintain the data models, analytical tools, and governance structures we establish during this engagement. This will empower the State to continue updating and refining the roadmap as conditions evolve.

This approach can empower stakeholders to assess trade-offs and anticipate challenges before making final decisions, enhancing both short-term resilience and long-term transformation effectiveness. The culmination of Phase 4 will result in a set of recommendations for sustainability, directly aligned with and incorporated into the statewide transformation roadmap. These recommendations offer a clear path to maintaining essential services and network adequacy, while positioning the system for future innovation and improvement.

In addition, we are able to work collaboratively with AHC and GMCB to co-develop a comprehensive sustainability and handoff plan. This plan will validate that the models and datasets created throughout the transformation process are maintained, updated, and effectively utilized by stakeholders beyond the initial engagement period. By establishing clear protocols, training, and ongoing support mechanisms, our team can help empower Vermont's leadership to leverage these resources for continuous improvement and informed decision-making across the health system.

By explicitly connecting financial assessment to strategic planning, Phase 4 confirms that the statewide transformation roadmap is actionable, sustainable, and positioned to deliver lasting improvements in access, quality, and health equity across the Vermont health system.

Our team serves as a strategic partner to Vermont's leadership, helping interpret results, weigh trade-offs, and build consensus on the most sustainable path forward. We plan to facilitate structured decision-making sessions with AHS, GMCB, and other stakeholders to rank transformation options using clear criteria for access, quality, and financial resilience. Our project leadership and advisors, drawing on experience advising other states on similar

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healthcare reforms, will translate complex modeling into data-driven policy implications. To help Vermont's leaders move from analysis to action, our team can apply Deloitte's Strategic Choice Structuring Methodology to translate complex findings into clear, evidence-informed decisions. This approach moves beyond data presentation to structured decision-making, helping AHS, GMCB, and hospital leaders weigh competing priorities such as financial sustainability, workforce resilience, and equitable access. Through facilitated working sessions, we can guide stakeholders in framing key issues as strategic choices, testing what must be true for each option to succeed, and using reverse engineering to identify underlying assumptions and dependencies. When choices involve subjective, such as whether to maintain certain service lines, regionalize care, or reallocate resources, our team can support Vermont leaders developing rank ordered options based on defined criteria, quantify likely impacts, and document the rationale behind each recommendation. This disciplined, transparent process positions Vermont's decision-makers to make, justify, and defend difficult decisions to external stakeholders, moving Vermont from insight to implementation.

For example, if Vermont faces a decision about whether to consolidate certain inpatient services at regional hubs or maintain them locally to preserve access, our team can use the Strategic Choice Structuring framework to evaluate the trade-offs. We help leaders assess how each option affects financial performance, patient outcomes, workforce stability, and travel times using both quantitative metrics and qualitative stakeholder input. Through reverse engineering, we can work backward from Vermont's goals for equitable access and sustainability to identify what conditions must be true for each option to succeed. This process allows policymakers to visualize the full impact of their decisions, compare alternatives on a common set of criteria, and select the approach that delivers the greatest value to Vermont's hospitals and communities.

Deliverables:

- **Statewide Transformation Roadmap Report:** Comprehensive strategic report, serving as the final Regional and Hospital Transformation Analytics Report, integrating financial, operational, innovation, and policy findings into a cohesive system-wide roadmap inclusive of:
 - **Facility Risk Profiles:** Individualized financial and operational summaries for each Vermont hospital, identifying strengths, vulnerabilities, and sustainability pathways
 - **Transformation Scenario Model:** Interactive toolset illustrating how different system configurations and innovation models (Hospital-at-Home, Telehealth, AI) impact access, cost, and quality. This will include a set of "ideal state" scenarios for the regionalization of specific service lines that align with statewide care transformation goals.
 - **Innovation Impact Report:** Quantitative evaluation of innovation scenarios demonstrating measurable outcomes and financial implications of technology-enabled care models
 - **Sustainability and Handoff Plan:** Implementation guidance, governance framework, and capacity-building plan to facilitate long-term adoption and maintenance of transformation tool

4 OUR EXPERIENCE

4.1 ORGANIZATION BACKGROUND AND HISTORY

Established in 1845, Deloitte is the largest and one of the most respected professional services firms in the world. For 175 years, clients have relied on us and our predecessor organizations, Deloitte Haskins & Sells and Touche Ross & Co., to meet business challenges and identify opportunities for innovation and transform the way they do business. Deloitte LLP, the US member firm of Deloitte Touche Tohmatsu Limited, was established as a limited liability

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partnership in 1994, with Deloitte Consulting LLP, one of its primary subsidiaries, following suit in 2003. We are one of the most trusted professional services firms in the world.

Figure 2: Organization Background and History

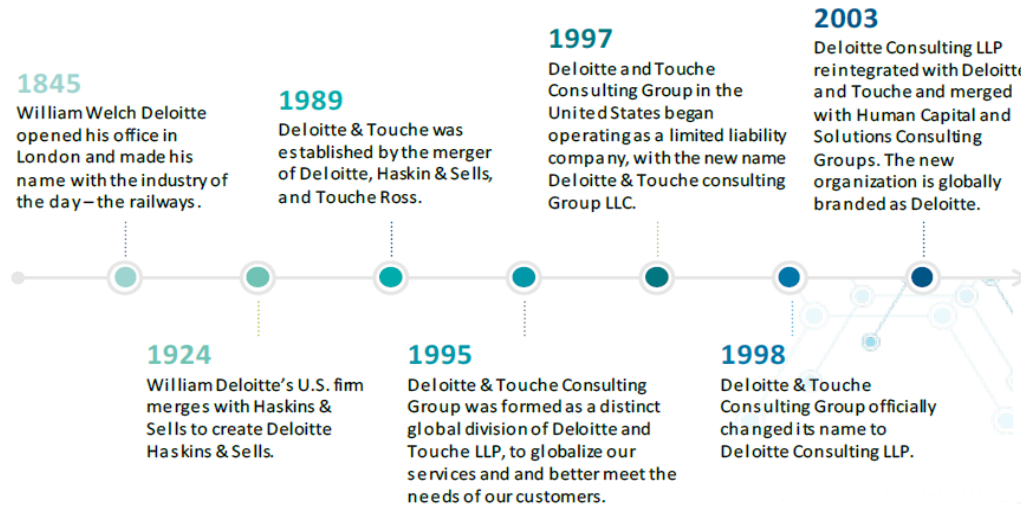
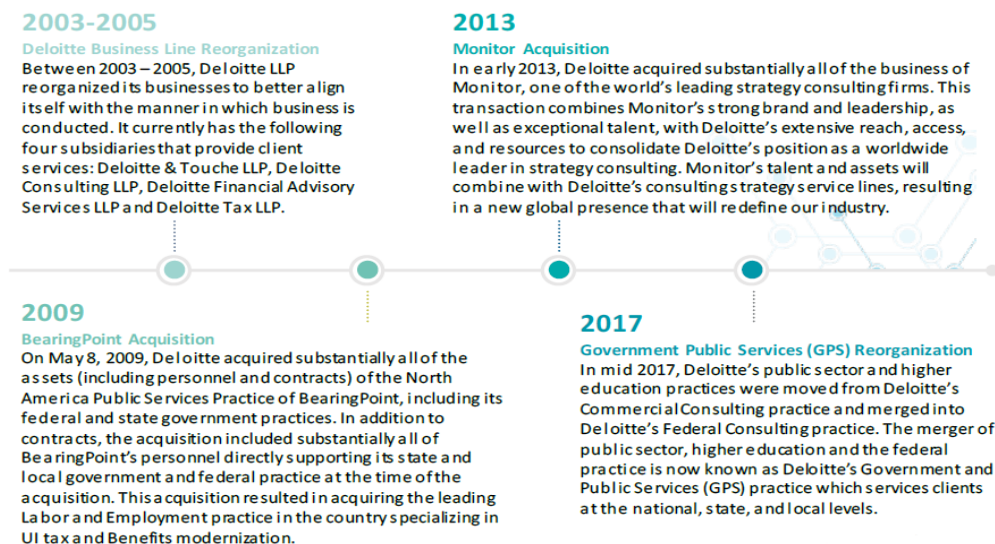


Figure 3: Deloitte Professional Services History



This extensive history has culminated in Deloitte Touche Tohmatsu Limited (DTTL) being the largest private professional services network in the world with the solutions and resources to help Deloitte member firm clients, our people, and sections of society achieve remarkable goals, solve complex problems, and make meaningful impact.

HOSPITAL TRANSFORMATION ANALYTICAL SUPPORT PROPOSAL

Today, we are the largest global consulting organization providing audit, tax, consulting, and financial advisory services. We have a proud history of assisting state governments with executing their strategic programs and delivering on their most important initiatives including modernization and replacement of old systems in highly visible areas such as health care, citizen benefits, core systems design, development and implementation, and cybersecurity with a specialization in Medicaid and analytics services. These projects include Medicaid Enterprise System (MES) modernization, core claims administration, medical management, Medicaid eligibility, hospital information systems, Electronic Health Records (EHR), Health Insurance Exchange (HIX), and Health Information Exchange (HIE) services. Over the past 50 years, our public sector practice has grown to more than 20,000 professionals. We have provided state and health and human services consulting services to 49 of 50 U.S. states, the District of Columbia, Puerto Rico, Canada, and the United Kingdom. Our team has deep health practice experience and perspective shared through client engagements, live events, signature reports, research publications, podcasts, and webcasts. We provide broad capabilities and significant resources across diverse disciplines to serve wide-ranging missions and deliver on assignments of any size. We understand each client is unique, and while we plan to apply our learning from past experiences, listening to the State's unique statements is key to comprehending their business needs and addressing their issues with objectivity.



4.2 VERMONT EXPERIENCE

Deloitte is proud to support ongoing work with the State of Vermont. This work enhances our understanding of Vermont's data landscape, privacy and security policies, and data governance standards. We are privileged to serve as a current vendor for the Agency of Digital Services (ADS) and the Department of Vermont Health Access (DVHA). Collaborating with the State is an honor, and we are dedicated to supporting the positive impacts and outcomes of these projects. Table 3 below outlines our current and past engagements with the State.

State Entity	Description of Solution and Services Provided	Dates
Agency of Digital Services (ADS) and Department of Vermont Health Access (DVHA)	• Providing a Software as a Service (SaaS) solution, including a Contractor-supported MDL and DAR Solution. • Developing, implementing, operating, and maintaining the solution, along with related IT systems and business consulting services. • Modernizing the MMIS ecosystem with advanced, modular, and scalable MDL and DAR capabilities. • Streamlining eligibility determination and case management processes. • Reducing the administrative workload on workers to focus on promptly processing benefits for Vermonters.	8/7/2023 - 4/6/2030
Agency of Digital Services (ADS) and Department of Vermont Health Access (DVHA)	• Providing MDWAS through a SaaS application called HealthMAP. • Designing, developing, implementing, supporting, and maintaining the solution, including related IT systems and business consulting services. • Achieving a Centers for Medicare & Medicaid Services (CMS) certification and facilitating a smooth transition. • Delivering a scalable and extensible data storage and reporting architecture. • Facilitating solution integration across the Vermont human services enterprise and supporting the State's ongoing transformation initiatives for MMIS modernization.	7/1/2024 - 12/31/2027
Department of Finance and Management	• Maintained an IT Retainer Services agreement with the Department of Finance and Management.	3/1/2019 - 2/28/2021

Table 3. Overview of past and current Deloitte projects with the State of Vermont

4.3 PROJECT EXPERIENCE

The following project experiences demonstrate Deloitte's proven capability and experience in providing similar, relevant work to the RFP requirements. While we have tailored our technical approach to Vermont's unique needs, these projects utilized a similar methodology and approach.

HOSPITAL TRANSFORMATION ANALYTICAL SUPPORT PROPOSAL

Distressed Provider Technical Assistance

Deloitte developed a series of Tableau dashboards, aggregating 6 separate data sources, to provide insights into hospital finances, operations, and clinical outcomes. In collaboration with New York State (NYS), our team developed a portfolio management approach that tracked 13 subsidies flowing to hospitals from across NYS to create a comprehensive view of subsidies going to the hospitals and the impact on their finances, operations, and clinical outcomes. Through these initial steps, the team is moving towards NYS' goal to reduce hospital reliance on subsidy payments and decrease the number of financially distressed hospitals.

Project Information

Organization	New York State (NYS) Department of Health (DOH)
Issue	NYS is subsidizing hospitals to help preserve access to care in both urban and rural communities. Without subsidies, many of these hospitals would be forced to close. NYS DOH works directly with acute care hospitals, and financially distressed hospitals (FDHs) in particular, to facilitate access to critical healthcare services, assessing an individual hospital's needs as well as the larger community's needs.
Deliverables	• Tableau Hospital Dashboards – The dashboards allow DOH to more easily review at a glance data from Institutional Cost Reports (ICR), and aggregate data across NYS hospital capacity, payor mix, CMS quality scores, and social vulnerability index scores in a consolidated place. • FDH Subsidy Tracker – This Tracker is used internally by NYS DOH to monitor payments and produce analyses of operational subsidies, capital funding, transformation investments, and federal funding that support NYS hospitals. • Regional Analyses – Regional analysis is used to evaluate community health needs, supply and demand of clinical services, payor mix, and migration patterns. This work provides DOH a deeper understanding of the changes and intricacies in the region, and how DOH can prudently allocate available subsidy dollars where it's needed most. • Hospital Financial Analyses – Facility analysis includes modeling impact to balance sheet, income statement, and cashflow statements under different subsidy allocations and assumptions. This allows DOH to gain foresight into performance trends.  <i>Example Screenshots from Tableau Hospital Dashboards</i>
Impact	The NYS DOH team utilizes the Hospital Dashboards and the FDH Subsidy Tracker, referring to them as the “gold standard” data sources. <i>The Hospital Dashboards</i> in Tableau provide DOH with user-friendly direct insights on individual hospitals and health systems, thus improving overall planning efficiency. The Dashboards also avail aggregate source data for one-off financial/operational analyses.

HOSPITAL TRANSFORMATION ANALYTICAL SUPPORT PROPOSAL

	<i>The FDH Subsidy Tracker</i> has streamlined DOH’s monitoring effort, as DOH previously relied on multiple Excel files that housed different subsidy data. Now, DOH uses the FDH Subsidy Tracker as a single source of truth for all data regarding the 13 subsidies provided to hospitals.
Maternal Health Services in Rural Health Facilities	
Deloitte was engaged to identify, analyze, and refine recommendations based on workgroup input to mitigate maternity and OB/GYN facility or service line closures in rural Oregon. As part of this work the team conducted external research to understand trends related to maternal health and OB/GYN care across the U.S. and evaluate what components of Sen. Wyden’s “Keeping Obstetrics Local Act” could be relevant for the state-level agency. The team also engaged with the Health Facility Closure Workgroup, comprised of Oregon Health Authority (OHA), other state agencies, and external stakeholders, to discuss this research as well as policy recommendations that would be most impactful for rural communities in Oregon.	
Project Information	
Organization	Oregon Health Authority (OHA)
Issue	Over the past 15 years, over 500 hospitals across the United States have closed their labor and delivery departments due to their inability to generate a profit. These closures have overwhelmingly impacted rural communities. In late 2023, the maternity ward at Saint Alphonsus Medical Center in Baker City, Oregon, closed requiring expectant mothers to drive at least 45 miles to the next closest facility expanding the number of women in Oregon who live in “Maternity Care Deserts.” In response, Senator Wyden of Oregon introduced the “Keeping Obstetrics Local Act,” which creates financial incentives for hospitals who keep their labor and delivery departments open. OHA sought guidance on which aspects of this proposed federal legislation would be most applicable to rural Oregon particularly specific alternative payment models described within.
Deliverables	• Full List of Policy Considerations – Our team supported the determination of 30+ policy recommendation ideas, which were narrowed down to 13 and presented to the OHA Director’s Office for further input. • Short-List of Policy Recommendations – Assessed the level of impact and effort for the 13 prioritized recommendations to determine the final list of 4 policy recommendations for the OHA Director’s Office with next steps for implementation. • Capacity Payment Policy Analysis - Overview and analysis of Capacity Payment Model for rural hospitals, challenges associated with the model, and an analysis of a Global Budget Model as an alternate solution. • Communications Protocol - A Communications Protocol was developed outlining the roles and responsibilities for how the state responds in the event of a health facility closure.
Impact	With external partner input, our team’s policy analysis, and other state research, 30+ policy recommendation ideas were narrowed down to 13, which were presented to the OHA Directors Office for further input. Recommendations were organized by category (e.g., workforce, financial/reimbursement, and regulatory/notifications) and included anticipated level of effort to actualize and who to further consult at OHA. This list was then narrowed down to four final recommendations for the OHA Director’s Office based on level of impact and level of effort. By bringing real-world experience and knowledge, our team helped OHA craft an unexpected and impactful path forward.

In addition to our collaboration with states, we have experience in the commercial healthcare industry integrating and transforming hospital functions to help achieve our clients’ goals.

HOSPITAL TRANSFORMATION ANALYTICAL SUPPORT PROPOSAL

Commercial Project Experience	Relevance to Vermont
Regional Health System Turnaround & Design	13-week project evaluating financially distressed hospitals, modeling scenarios for sustainable operations, and creating ROI-based turnaround and build-vs-buy strategies.
Nebraska Medicine Integration & Transformation	Risk-based diligence and hospital integration for an academic medical center; financial turnaround and operating model design.
Dignity Health Facility Integration	Consolidation analysis of two hospitals to reduce losses and improve coordination.
OSF HealthCare and Ottawa Regional Medical Center	Day 1 blueprinting across 17 functions for hospital integration; rural health access relevance.
Sanford Health & Good Samaritan Society	Integration of 200+ post-acute facilities; synergy modeling and \$50M margin improvement plan, aligning with Vermont's regional sustainability focus.

5 OUR DELOITTE TEAM

5.1 WHO WE ARE

Our proposed team has been hand-selected for this engagement, bringing together leaders who share a deep professional and personal commitment to building thriving, sustainable healthcare ecosystems. Each member was chosen not only for their depth of knowledge and experience in health system transformation, data analytics, and public policy, but also for their shared passion for helping states like Vermont create health systems that are equitable, resilient, and future-ready. Together, we unite decades of experience in Medicaid analytics, hospital strategy, and state health leadership with a common goal: help Vermont strengthen the foundation of care across its entire health ecosystem.

Our team includes former state health officials, clinicians, data scientists, and transformation strategists who have dedicated their careers to improving how care is financed, delivered, and measured. They have guided states through complex reforms, designed performance frameworks that sustain hospital systems, and built analytic models that have informed healthcare policy decisions across the country. Their experience spans the full continuum of care from primary and preventive services to acute and specialty care, and they are grounded in an understanding of the operational, financial, and human realities that shape system-wide change.

We approach this work as co-collaborators in Vermont's transformation journey, motivated by a shared belief that data-driven insights and community-focused strategy can build a more resilient health system for every Vermonter. Many of us have worked alongside states facing similar crossroads, and we recognize both the complexity and the opportunity this moment represents. Our team brings not just technical excellence, but a genuine sense of purpose to help Vermont design a healthcare system that thrives economically, delivers exceptional outcomes, and reflects the values of the people it serves.

5.2 OUR TEAM STRUCTURE

Our project team is prepared to start immediately, combining strategic leadership, operational efficiency, and expert advisory support. Our solutions are both technically sound and aligned with business objectives.

Executive Leadership Team	Core Operational Team
 Greg Spino <i>Executive Sponsor</i>	 Chris Lattanzio <i>Technical Delivery Lead</i>
 Keara Klinepeter <i>Engagement Lead</i>	 David Goott <i>Data Engineer</i>
Strategic Advisors	
 Felix Matthews, M.D. <i>Clinical and Operational Health System Advisor</i>	 Athnee Ononiewu <i>Senior Data Analyst</i>
 Atresha Whitmore <i>Vermont Account Advisor</i>	 Hannah Wind <i>Data Analyst</i>
 Scott Bonin <i>Hospital Financial Advisor</i>	

The Executive Leadership Team sets strategic direction and establishes alignment with client goals and Deloitte standards. The Core Operational Team executes plans by designing and implementing data and technology solutions. Our Strategic Advisors contribute specialized knowledge to keep technical decisions aligned with broader operations and industry needs and will be available to meet with Vermont monthly based on the needs of the project. Resumes of our proposed team are included below for a more detailed view of the robust skillsets this team will bring.

5.3 RESUMES

Greg Spino

Account Executive (Key)



SUMMARY OF EXPERIENCE

The Account Executive is responsible for managing client relationships and supporting project success and Internal alignment and milestone tracking.

Gregory Spino is a Director in Deloitte's Public Sector State practice, specializing in Medicaid Data Management and Analytics. He has led the design and implementation of large-scale data solutions, including enterprise data warehouses and analytic platforms, with successful CMS-certified projects in Massachusetts and Wyoming. Greg's experience spans performance management, provider platforms, opioid reporting, payment reform, and population health. He is known for integrating complex data sources, enabling self-service analytics, and driving data-informed organizational change to meet Medicaid program goals. He serves as the Account Manager for the MDWAS program in Vermont.



RIGHT FIT FOR VERMONT

Greg brings the following capabilities and experience to this project:

- Extensive Leadership in Medicaid Data Analytics, Public health and Warehousing**
 With over **19 years of experience** designing and delivering **Medicaid enterprise data analytics solutions**, Greg has led large-scale implementations like **Massachusetts' and Wyoming's Medicaid data warehouses**, both of which achieved **CMS certification**—a testament to his ability to deliver compliant, high-impact solutions.
- Knowledge of Complex Data Integration and Health Outcomes Analytics**
 Greg has deep experience integrating **claims, provider, and population health data** from multiple sources, including **All-Payer Claims Databases (APCDs)** across six states—skills that are directly applicable to VHIE's mission of unifying health data for improved care coordination and outcomes.
- Strategic Vision and Agile Project Delivery**
 As a **PMI Agile-certified project manager**, Greg has successfully led **multi-vendor teams** through complex implementations using Agile and DevOps methodologies, enabling **rapid, iterative delivery** of analytics platforms that support evolving Medicaid and public health needs.
- Success in Supporting Alternative Payment Models and Public Health Reporting**
 Greg has developed analytics to support **bundled payments, medical homes, opioid reporting, and performance management**, aligning with VHIE's goals of enabling value-based care and data-driven public health strategies.
- Strong Stakeholder Engagement and Executive Leadership**
 In his role as **Account Executive for the Wyoming Department of Health**, Greg served as the primary liaison between client leadership and project teams, demonstrating his ability to align technical solutions with strategic goals—critical for fostering collaboration and long-term success in a statewide HIE environment.

Keara Klinepeter

Engagement Lead (Key)



SUMMARY OF EXPERIENCE

The Engagement Lead is responsible for overseeing resource allocation, driving team collaboration, and navigating risk mitigation.

Ms. Klinepeter works nationally with Health & Human Services Agencies to preserve and expand access to acute care. As the former Acting Secretary of Health in Pennsylvania, Keara brings deep state government experience with emphasis on alternative payment and delivery models, strategic planning, and enterprise transformation.



RIGHT FIT FOR VERMONT

Keara brings the following capabilities and experience to this project:

- Leads a 5-year, \$37.5M project with a large state **identifying the root causes of financial distress at their hospitals** and developing financial and policy solutions. Her and her team's work is at the intersection of Medicaid, public health, hospitals, and commercial payers, which has honed her understanding of how these complex systems interact and can be leveraged in tandem to achieve financially sustainable outcomes for providers and improved quality of care for patients. This work required a detailed understanding of hospital finances, operations, provider incentives, regional planning, alternative delivery and payment models, and more.
- Directed Pennsylvania's COVID-19 response as Acting Secretary of Health during the largest case surge, **proactively mitigating healthcare capacity risks**, expanding free state-run testing, issuing innovative hospital guidance, securing \$250M for frontline workers, and advocating for public health infrastructure funding, resulting in sustained access to care and enhanced statewide pandemic resilience.
- **Directed rural health innovation** as Director of Rural Health Innovation for the PA Rural Health Model, overseeing transformation planning for ~25 rural hospitals by leading a team of 8, collaborating closely with hospital leadership to develop and implementing clinical, operational, financial, and community-based strategies, and securing technical assistance valued at ~\$300K—resulting in strengthened rural hospital sustainability and improved health outcomes for rural communities.

Chris Lattanzio

Technical Delivery Lead (Key)



SUMMARY OF EXPERIENCE

The Technical Delivery Lead is responsible for guiding the technical direction of the project, validating high-quality solutions and mentoring the team.

Chris Lattanzio is a Manager in Deloitte's Health Care & Life Sciences practice. He brings over ten years of specialized experience in delivering comprehensive financial advisory services to hospitals and health systems. He has effectively led clients through complex mergers and acquisitions, developed and executed hospital turnaround strategies, and conducted in-depth operational assessments. Chris's knowledge and experience spans the full deal lifecycle including due diligence, advanced financial modeling, synergy identification, and integration planning. His strategic insights and hands-on approach have consistently enabled clients to achieve their financial and operational objectives in an evolving healthcare landscape.



RIGHT FIT FOR VERMONT

Chris brings the following capabilities and experience to this project:

- Lead a team of 15+ delivering advisory services for the **State Department of Health's \$8B oversight of 70+ financially distressed hospitals**—including funding allocation decisions, financial and operational analysis, performance improvement, strategic planning, grant management, stakeholder engagement, and development of analytical tools.
- Provided **physician enterprise, financial diligence, and modeling services** for a \$7.5B health system's \$2.4B acquisition of three hospitals in the Southeast.
- Delivered end-to-end M&A services for a \$500M carve-out acquisition of three hospitals, increasing organizational size by ~30%, including quality of earnings, five-year projections, and capital planning.
- Conducted performance gap analysis for a \$4.5B health system, **uncovering margin improvement opportunities and recommending cost structure and sizing adjustments** based on revenue and benchmark analysis.
- Supported a \$1.6B NYC health system in **developing a stabilization plan to reduce State subsidy reliance**, including \$100M capital plan review, operational improvement assessment, and five-year strategic options such as M&A and service line rationalization.

David Goott

Data Engineer (Key)



SUMMARY OF EXPERIENCE

The data engineer works as part of our delivery team supporting the oversight of data integration, modeling, and visualization.

David Goott is a Senior Consultant with over 4 years of experience consulting to State governments. He brings specialized experience in data analysis, data visualization, and data storytelling regarding hospital and healthcare analytics. His experience includes guiding states through the preparation for and implementation of a new government division designated to operate a State-based Exchange, and developing a comprehensive regional analysis focused on patient migration trends and identifying potential service duplications.



RIGHT FIT FOR VERMONT

David brings the following specific experience to this project:

- Provided data analysis and data visualization to uncover insights regarding patient migration, hospital performance, regional trends, and service optimizations.
- Uncovered insights to support State decision-making regarding future subsidy funding determinations.
- Developed a Tableau dashboard to group over 100 State hospitals into archetypes based on financial, operational, quality, and strategic metrics to support decision making and strategic planning.
- Developed and remediated Tableau dashboards tracking COVID-19 health data to improve the user experience and adhere to accessibility standards.
- Supported the stand-up of a new State-based Exchange for health insurance that utilizes private sector entities and a public consumer portal for enrollment in health plans.

Athnee Ononiewu

Senior Data Analyst



SUMMARY OF EXPERIENCE

The Senior Data Analyst works as part of our delivery team and is responsible for overseeing and executing day-to-day tasks related to data aggregation and synthesis.

Athnee Ononiewu is a dynamic and agile professional with over 10 years of experience in finance, accounting, risk management and project management. She has a proven track record of delivery excellence in enterprise performance, optimization, and automation of business processes towards creating a healthy and stable business operation.



RIGHT FIT FOR VERMONT

Athnee brings the following capabilities and experience to this project:

- Analyzed multi-year appropriations in relation to fund allocation and expenditures for a public agency with several on-site facilities as they sought to obtain comprehensive analysis into payroll cost requirements to support security services. Performed detailed analysis of historical financial data over multiple periods to identify costs and spend trends, and prepared cost projections to estimate the future costs for client's expenses
- Collaborated with a State's Health and Human Service agency to design modern data architecture and to develop a data warehouse to improve operational efficiency. Led stakeholder meetings to validate the accuracy and completeness of data mapping analysis. Identified and mapped underlying data files (files, tables, reports, etc.) under source systems and provided strategic support towards automation, and data transformation
- Conducted a modernization assessment and evaluated technology solutions for a State's Health and Human Service agency to implement an end-to-end transformation of the processes under the CFO departments (Accounting, Actuarial, Budget, Forecasting, and Provider Finance) to streamline and create efficiencies.

Hannah Wind

Data Analyst



SUMMARY OF EXPERIENCE

The data analyst works as part of our delivery team and is responsible for day-to-day tasks related to data aggregation and synthesis.

Hannah Wind is an Analyst in Deloitte's Health & Human Services practice with data analytics experience across platforms such as Excel, Tableau, Arena, and Minitab. She has consolidated and analyzed thousands of data points to identify solutions for supply chain challenges, presented actionable insights to senior management, and provided predictive recommendations for manpower capacity across various operations. Collaborating closely with program management, supervisors, and manufacturing engineers, Hannah has driven improvements in operational efficiency, including queue time reduction and process optimization. She also led a 5S Lean event for a team of 40, successfully standardizing and organizing a manufacturing space, consistently supporting clients in achieving efficiency and strategic goals in dynamic environments.



RIGHT FIT FOR VERMONT

Hannah brings the following capabilities and experience to this project:

- Worked with a local New England non-profit organization to determine anticipated funding shortfalls and review current financial data.
- Developed process maps and data flow diagrams to establish current state of internal reporting and analyzed data consumption patterns and provided future recommendations for a global investment managing firm
- Created a central database consisting of updated analyses, reference material, and training documents to communicate improved best practice standards for a large-scale distributor

Felix Matthews, M.D.

Healthcare Strategy & Clinical Advisor



SUMMARY OF EXPERIENCE

Dr. Matthews has 20+ years of experience in medical practice and health care consulting. Felix has led strategic transformation efforts for major academic health systems, integrated delivery networks, and specialty cancer centers. He works closely with executive leaders and Board members to shape the future of health care organizations, while also accompanying clients throughout their transformation journey. In this capacity, he understands the challenges of prioritization, strategic choices, stakeholder engagement, and change management. Felix led the evaluation of overall hospital and nursing home performance and the performance of financially distressed hospitals for NY since 2022, as well as several regional healthcare ecosystem assessments.

Scott Bonin

Advisor



SUMMARY OF EXPERIENCE

Scott brings 17+ years of experience working with Health Care and Life Science clients. Scott advises his clients through challenging financial situations by means of developing robust economic models, advising on policy changes, and identifying and overcoming strategic threats to core operation. He provides a unique perspective through a nearly 2-decade history of working with not-for-profit health systems and state-owned medical centers. Scott is passionate in providing informed decisions to enhance the delivery of healthcare, which often involves leading executives and senior management through challenging environments.

Atresha Whitmore

Advisor



SUMMARY OF EXPERIENCE

Atresha brings over 14 years of experience helping the government clients improve customer experiences through strategic program support. Ms. Whitmore has 10+ years of conducting strategic program management and delivering complex transformation portfolios across the federal government. Atresha has worked on capability development, stakeholder management, risk and dependency management, and has collected leading practices throughout.

6 REFERENCES

Reference 1: State of New York

Deloitte collaborated with New York State (NYS) to develop a portfolio management approach that **tracked 13 subsidies flowing to hospitals** from across NYS to create a comprehensive view of subsidies going to the hospitals. Our team also **developed a series of Tableau dashboards** for NYS to utilize in daily analysis. Both efforts significantly improved NYS's insight into how subsidies affect hospital finances, operations, and clinical outcomes. This enabled NYS to make more informed decisions, advancing their goal to **reduce hospital reliance on subsidies and strengthen financial stability** across the entire health system.

Project Information

Location	New York
Dates of Contract Period	June 2024 — May 2029
Contact Name / Title	Collin Gulczynski – Deputy Division Director
Contact Email Address	Collin.Gulczynski@health.ny.gov
Contact Phone Number	518-474-2550

Reference 2: State of Florida Department of Health

Deloitte's extensive experience serving state public health departments in tandem with a thorough understanding of the Centers for Disease Control and Prevention's (CDC) modernization goals positioned us to provide tailored solutions to the Florida Department of Health (FDOH) such as **gap analysis, tailored roadmap**, and workplan to launch their modernization journey. **Centering key stakeholders across the agency** through information gathering sessions (interviews, surveys, focus groups, visioning labs) allowed FDOH to have a holistic understanding of their data, technology, and governance ecosystems, the workforce required to meet the goals and objectives of Data Modernization Initiative (DMI), and an overall plan for dismantling data and application silos. Bringing forth these threads created an enterprise DMI vision the agency could work toward and a consensus driven approach across stakeholders.

Project Information

Location	Florida
Dates of Contract Period	August 2023 – June 2024
Contact Name / Title	Pura Ahler – Deputy Director
Contact Email Address	Pura.Ahler@flhealth.gov
Contact Phone Number	850-245-4239

Reference 3: CommonSpirit Health System

Deloitte works with CommonSpirit for identification of direct financial savings and indirect savings, identification of capital investments and IT investments needed, creation of national, **regional governance model**, 5-year financial proforma creation, board document creation, project communication assistance, workforce transformation, operating model re-design, and overall project management / change management. Our team assessed and integrated 3 regional Management Services Organizations (MSO) into one State-focused MSO with ambition to scale services nationally. We also assessed population health activities and capabilities **across 7 regions** in the US to create the following:

1. National Population Health structure for selected core activities
2. **Enhanced Regional Population Health Structure** with more focused task related to **regional activities and oversight**
3. Hospital specific population health structure for physician groups to maintain required service where local services were needed versus regional and national services

Project Information

Location	Nationwide
Dates of Contract Period	January 2021 – Present
Contact Name / Title	Derek Novak – Senior VP Population Health
Contact Email Address	derek.novak@commonspirit.org
Contact Phone Number	515-954-4373

7 EXCEPTIONS

Deloitte Consulting LLP (“Deloitte Consulting”) is pleased to submit this proposal to State of Vermont, the Agency of Human Services – Office of Healthcare Reform, (hereinafter the “State”) in response to its Request for Proposal (the “RFP”) for Hospital Transformation Analytical Support. We believe, however, that certain of the provisions included in the Standard Contract Form for Technology Services, and its attachments (the “Proposed Terms”) should be modified or clarified for this engagement.

Pursuant to RFP Sections 3.7 and 3.9, our proposal is being made subject to the condition that Deloitte Consulting and the State subsequently reach and enter into a mutually agreeable definitive written agreement for the proposed services. As such, we propose that the parties agree to consider leveraging previous negotiations, including substantively similar terms and conditions in accordance with previous or existing contracts between the State of Vermont and Deloitte Consulting, such as the Medicaid Data Lake and Data Analytics and Reporting Solution, Contract #45517. If we are awarded this agreement, we agree to negotiate in good faith to reach an agreement that is mutually satisfactory to both State and Deloitte Consulting.

We anticipate further discussion during negotiations to include the following sections in the Proposed Terms.

Insurance (Attachment C, Section 8; (Professional Liability and Cyber Liability Insurance Coverage (Standard Contract for Services)) - Deloitte Consulting may request some clarifying modifications to the insurance requirements to make such requirements consistent with the insurance that we maintain, which we believe is consistent with insurance maintained by other large professional services firms and as mutually agreed upon by Deloitte Consulting LLP and the State of Vermont, Agency of Digital Services as part of the Medicaid Data Lake and Data Analytics and Reporting Solution Contract, Contract # 45517 and Medicaid Analytics Data Warehouse Solution Contract, Contract # 47702.

We would also like to discuss and propose additional terms, such as a commercially reasonable limitation on liability. The following is the limitation on liability in the Medicaid Data Lake and Data Analytics and Reporting Solution contract, Contract # 45517*:

- (a) CONTRACTOR’S LIABILITY FOR DAMAGES TO THE STATE ARISING OUT OF THE SUBJECT MATTER OF THIS CONTRACT SHALL NOT EXCEED THREE TIMES THE MAXIMUM AMOUNT PAYABLE UNDER THIS CONTRACT. LIMITS OF LIABILITY FOR STATE CLAIMS SHALL NOT APPLY TO STATE CLAIMS ARISING OUT OF: (A) CONTRACTOR’S OBLIGATION TO INDEMNIFY THE STATE FOR (i) INFRINGEMENT BY ANY MATERIALS PROVIDED BY CONTRACTOR OF THIRD-PARTY PATENTS, COPYRIGHTS, OR TRADEMARKS; (ii) VIOLATION OF LAWS APPLICABLE TO CONTRACTOR IN ITS PERFORMANCE OF OBLIGATIONS HEREUNDER; (iii) FINES AND PENALTIES IMPOSED ON THE STATE BY A GOVERNMENT ENTITY TO THE EXTENT RESULTING FROM THE CONTRACTOR’S VIOLATION OF ANTI-BRIBERY AND ANTI-CORRUPTION LAWS IN ITS PERFORMANCE OF THE SERVICES; (iv) CLAIMS BROUGHT AGAINST THE STATE BY ANY PERSONNEL OF CONTRACTOR PERFORMING SERVICES HEREUNDER FOR EMPLOYMENT BENEFITS OR EMPLOYMENT COMPENSATION, IN EACH CASE FOR WHICH CONTRACTOR IS RESPONSIBLE AND HAS FAILED TO PAY, EXCEPT TO THE EXTENT THAT SUCH CLAIM RESULTS FROM ACTS OR OMISSIONS OF THE STATE; (v) CLAIMS BROUGHT AGAINST THE STATE BY ANY CONTRACTOR SUBCONTRACTOR PERFORMING A PORTION OF THE SERVICES HEREUNDER FOR PAYMENT OF ITS FEES TO THE EXTENT CAUSED BY CONTRACTOR’S FAILURE TO PAY SUCH FEES; (vi) CLAIMS AGAINST THE STATE BASED UPON CONTRACTOR’S RECKLESSNESS, BAD FAITH, OR INTENTIONAL MISCONDUCT; (vii) ANY THIRD PARTY CLAIMS FOR VIOLATIONS OF THE FREEDOM OF INFORMATION ACT OR OTHER SIMILAR STATE LAW RESULTING FROM CONTRACTOR’S CLAIM THAT CONTRACTOR CONFIDENTIAL INFORMATION IS EXEMPT FROM DISCLOSURE UNDER SUCH LAWS; AND (viii) VIOLATIONS BY CONTRACTOR OF ATTACHMENT D, SECTION 6, SECURITY OF STATE INFORMATION, OF ATTACHMENT; (B) CONTRACTOR’S CONFIDENTIALITY OBLIGATIONS TO THE STATE; (C) PERSONAL INJURY OR DAMAGE TO REAL OR PERSONAL PROPERTY; (D)

HOSPITAL TRANSFORMATION ANALYTICAL SUPPORT PROPOSAL

CONTRACTOR'S GROSS NEGLIGENCE, FRAUD OR INTENTIONAL MISCONDUCT; OR (E) VIOLATIONS OF THE STATE OF VERMONT FRAUDULENT CLAIMS ACT. IN NO EVENT SHALL THIS LIMIT OF LIABILITY BE CONSTRUED TO LIMIT CONTRACTOR'S LIABILITY FOR THIRD PARTY CLAIMS AGAINST THE CONTRACTOR WHICH MAY ARISE OUT OF CONTRACTOR'S ACTS OR OMISSIONS IN THE PERFORMANCE OF THIS CONTRACT.

NEITHER PARTY SHALL BE LIABLE TO THE OTHER FOR ANY INDIRECT, INCIDENTAL OR SPECIAL DAMAGES, DAMAGES WHICH ARE UNFORESEEABLE TO THE PARTIES AT THE TIME OF CONTRACTING, DAMAGES WHICH ARE NOT PROXIMATELY CAUSED BY A PARTY, SUCH AS LOSS OF ANTICIPATED BUSINESS, OR LOST PROFITS, INCOME, GOODWILL, OR REVENUE IN CONNECTION WITH OR ARISING OUT OF THE SUBJECT MATTER OF THIS CONTRACT.

The provisions of this Section shall apply notwithstanding any other provisions of this Contract or any other agreement.

*Note changes required for differences in contract section references.

8 ATTACHMENTS

PRICE SCHEDULE

A. Fixed Price Deliverables:

Deliverable Description	Fixed Price
Data Integration & Analysis Plan	\$435,938
Hospital & Regional Access Modeling	\$435,938
Market & Financial Analysis	\$508,594
Evidence-Based Benchmarking Evaluation	\$363,281
Transformation Scenario Model	\$435,938
Final Regional and Hospital Transformation Analytics Report	\$145,313
Total Project Cost	\$2,325,000

Name of Bidder: Gregory Spino

Signature of Bidder: *Gregory Spino*

Date: October 28, 2025

CERTIFICATE OF COMPLIANCE

For your bid to be considered valid, this form must be completed in its entirety, executed by a duly authorized representative of the bidder, and submitted as part of the response to the proposal.

- A. **NON COLLUSION:** Bidder hereby certifies that the prices quoted have been arrived at without collusion and that no prior information concerning these prices has been received from or given to a competitive company. If there is sufficient evidence to warrant investigation of the bid/contract process by the Office of the Attorney General, bidder understands that this paragraph might be used as a basis for litigation.
- B. **CONTRACT TERMS:** Bidder hereby acknowledges that is has read, understands and agrees to the terms of this RFP, including Attachment C: Standard State Contract Provisions, and any other contract attachments included with this RFP.
- C. **WORKER CLASSIFICATION COMPLIANCE REQUIREMENT:** In accordance with Section 32 of The Vermont Recovery and Reinvestment Act of 2009 (Act No. 54), the following provisions and requirements apply to Bidder when the amount of its bid exceeds \$250,000.00.

Self-Reporting. Bidder hereby self-reports the following information relating to past violations, convictions, suspensions, and any other information related to past performance relative to coding and classification of workers, that occurred in the previous 12 months.

Summary of Detailed Information	Date of Notification	Outcome
None		

Subcontractor Reporting. Bidder hereby acknowledges and agrees that if it is a successful bidder, prior to execution of any contract resulting from this RFP, Bidder will provide to the State a list of all proposed subcontractors and subcontractors' subcontractors, together with the identity of those subcontractors' workers compensation insurance providers, and additional required or requested information, as applicable, in accordance with Section 32 of

The Vermont Recovery and Reinvestment Act of 2009 (Act No. 54), and Bidder will provide any update of such list to the State as additional subcontractors are hired. Bidder further acknowledges and agrees that the failure to submit subcontractor reporting in accordance with Section 32 of The Vermont Recovery and Reinvestment Act of 2009 (Act No. 54) will constitute non-compliance and may result in cancellation of contract and/or restriction from bidding on future state contracts.

D. Executive Order 05 – 16: Climate Change Considerations in State Procurements Certification

Bidder certifies to the following (Bidder may attach any desired explanation or substantiation. Please also note that Bidder may be asked to provide documentation for any applicable claims):

1. Bidder owns, leases or utilizes, for business purposes, space that has received:

- ☐ Energy Star® Certification
☒ LEED®, Green Globes®, or Living Buildings ChallengeSM Certification
☐ Other internationally recognized building certification:

The building is LEED Platinum and WELL Gold certified.

2. Bidder has received incentives or rebates from an Energy Efficiency Utility or Energy Efficiency Program in the last five years for energy efficient improvements made at bidder's place of business. Please explain:

Achievement of Leadership in Energy & Environmental Design (LEED) certification for new facilities and retrofits totaling over 1.8 million square feet of LEED certified space.

3. Please Check all that apply:

- ☒ Bidder can claim on-site renewable power or anaerobic-digester power ("cow-power"). Or bidder consumes renewable electricity through voluntary purchase or offset, provided no such claimed power can be double-claimed by another party.
☐ Bidder uses renewable biomass or bio-fuel for the purposes of thermal (heat) energy at its place of business.
☒ Bidder's heating system has modern, high-efficiency units (boilers, furnaces, stoves, etc.), having reduced emissions of particulate matter and other air pollutants.
☒ Bidder tracks its energy consumption and harmful greenhouse gas emissions. What tool is used to do this? Passive House
☒ Bidder promotes the use of plug-in electric vehicles by providing electric vehicle charging, electric fleet vehicles, preferred parking, designated parking, purchase or lease incentives, etc..
☐ Bidder offers employees an option for a fossil fuel divestment retirement account.
☒ Bidder offers products or services that reduce waste, conserve water, or promote energy efficiency and conservation. Please explain:

Deloitte (US) has recycling programs in most of its offices to reduce waste and make positive impact on the environment.

In addition to recycled 800 mattresses resulting in 31.2 tons diverted from landfills by collaborating with a recycling facility in

Virginia for their reuse: Inner springs will be re-melted and poured into new steel for construction material foam will be ground

and used to create carpet padding. The fabric will be used to make animal bedding and cloth for oil spill containment Wood

framing will be chipped and used as bedding, mulch, or biomass fuel All guestroom items were donated to Habitat for Humanity

or reuse locally.

4. Please list any additional practices that promote clean energy and take action to address climate change:

Construction

Deloitte (US) incorporates sustainable design guidelines in the design and construction of our own workplaces, i.e., new and renovated offices in our US real estate portfolio. From site/building selection and design through lease-end operation, we aim to minimize our footprint's impact on the environment by reducing energy and water utilization; thoughtfully selecting and responsibly sourcing materials, both natural and manmade; ensuring indoor air quality; and reducing waste. To that end, we have adopted the following sustainability design guidelines, which apply to all US workplace design projects regardless of size or scope

Office building/site

Focus on newer, more efficient buildings. These buildings save energy, water, and resources, generate less waste, and support human health. We recognize that the ability to deliver on this varies by city/market and the options available for the timing needed.

Materials, finishes, furniture

Select materials - including furniture - with a high percentage of recycled content, where viable. Note: Many products contain recycled content; there is an opportunity to limit or eliminate the use of non-sustainable materials (e.g., chrome), especially in furniture/upholstery. Carefully consider a supplier's distribution location when selecting materials; we prefer materials (manmade and natural) to be regionally sourced, where possible, to reduce the fuel needed for transport and to promote local economies. Specify low/non-VOC emitting materials (e.g., paint, adhesives, carpeting).

Water

Specify low-flow fixtures/equipment for amenity spaces and restrooms.

Lighting

Design floor plans that optimize daylight harvesting and reduce electrical lighting.

E. Executive Order 02 – 22: Solidarity with the Ukrainian People

- ☒ By checking this box, Bidder certifies that none of the goods, products, or materials offered in response to this solicitation are Russian-sourced goods or produced by Russian entities. If Bidder is unable to check the box, it shall indicate in the table below which of the applicable offerings are Russian-sourced goods and/or which are produced by Russian entities. An additional column is provided for any note or comment that you may have.

Provided Equipment or Product	Note or Comment

F. Certification Regarding Use of Contract Funds for Lobbying - Applicable to contracts over \$100,000.00 when federal monies are involved - this clause must be included in all subcontracts over \$100,000.00.

1. The prospective contractor certifies, to the best of his or her knowledge and belief, under the penalties of perjury under the laws of the State of Vermont and the United States that on behalf of the person, firm, association, or corporation he or she represents, that:

a. No Federal appropriated funds have been paid or will be paid, by or on behalf of the undersigned, to any person for influencing or attempting to influence an officer or employee of any Federal agency, a Member of Congress, an officer or employee of Congress, or an employee of a Member of Congress in connection with the awarding of any Federal contract, the making of any Federal grant, the making of any Federal loan, the entering into of any cooperative agreement, and the extension, continuation, renewal, amendment, or modification of any Federal contract, grant, loan, or cooperative agreement.

b. If any funds other than Federal appropriated funds have been paid or will be paid to any person for influencing or attempting to influence an officer or employee of any Federal agency, a Member of Congress, an officer or employee of Congress, or an employee of a Member of Congress in connection with this Federal contract, grant, loan, or cooperative

agreement, the undersigned shall complete and submit Standard Form-LLL, "Disclosure Form to Report Lobbying," in accordance with its instructions.

2. This certification is a material representation of fact upon which reliance was placed when this transaction was made or entered into. Submission of this certification is a prerequisite for making or entering into this transaction imposed by 31 U.S.C. 1352. Any person who fails to file the required certification shall be subject to a civil penalty of not less than \$10,000 and not more than \$100,000 for each such failure.

3. The prospective contractor also agrees that they shall require that the language of this certification be included in all lower tier subcontracts, which exceed \$100,000 and that all such recipients shall certify and disclose accordingly.

Bidder Name: Deloitte Consulting LLP Contact Name: Gregory Spino

Address: 115 Federal Street Fax Number: N/A

Winthrop Center Floors 12-15, Telephone: +1-617-437-3483

Boston, MA 02110 E-Mail: gspino@deloitte.com

By: *Gregory Spino* Name: Gregory Spino
Signature of Bidder (or Representative) (Type or Print)

END OF CERTIFICATE OF COMPLIANCE